

Gout

A summary based on NICE guidance (NICE 2022, NG219)

Red Whale



GEMS

Guidelines & Evidence Made Simple

Diagnosis and investigations

Suspect gout: Rapid-onset (often overnight) severe pain, redness and swelling in one or both 1st metatarsophalangeal (MTP) joints OR presence of tophi.

Consider gout: Rapid-onset severe pain, redness and swelling in joints **other than 1st MTP joint** OR in those with chronic inflammatory joint pain.

Differentials: **SEPTIC ARTHRITIS (admit!), pseudogout, other inflammatory arthritis.**

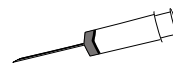
Measure serum uric acid (SUA) in people you consider or suspect to have gout.

If **SUA ≥ 360 micromol/l**, this confirms the clinical diagnosis.

If **SUA < 360 micromol/l** and gout strongly suspected, repeat at least 2 weeks after the flare has settled.

Diagnosis uncertain? Consider joint aspiration (with microscopy of aspirate), or consider imaging with Xray, ultrasound or dual-energy CT if aspiration cannot be carried out.

At Red Whale, we would also do U&E, lipids, HbA1c and LFT if no recent results (see full Gout article).



Managing flares: choose from:

Colchicine **OR** NSAID **OR** Corticosteroid

+/- PPI

ICE!

How to choose? Tailor choice of drug depending on: comorbidities, other drugs, patient preference.

...And, if none are suitable...

Consider intra-articular or IM corticosteroid (off-label).

NICE does not specify specific doses (see BNF).

Give information/support, including:

- Symptoms and signs.
- Causes of gout.
- That gout progresses without treatment.
- Explain/address risk factors (genetics, BMI, drugs, comorbidities, e.g. hypertension or CKD).
- How to manage flares.
- That gout is a lifelong condition benefitting from urate-lowering therapy to eliminate crystals.
- See resources box in main article for a useful leaflet from Versus Arthritis.

Info

Follow-up after flare:

Consider follow-up to:

- Measure urate level.
- Review medications.
- Assess lifestyle and comorbidity: CVD risk & CKD.
- **Do not** recommend a specific diet.
- Discuss that excess body weight/obesity, or excess alcohol, may exacerbate gout.
- Discuss pros/cons of **urate-lowering therapies (ULT)**.



Urate-lowering therapies (ULT): shared decision and targets

Offer ULT if: multiple flares, CKD 3–5, diuretic therapy, tophi, chronic gouty arthritis.

Discuss ULT after a first attack of gout (if not included in the groups above).

Explain that ULT is usually a life-long treatment.

When? Start 2–4w after flare settles, but *can be started during a flare* if attacks frequent.

Treat-to-target strategy: aim for **SUA ≤ 360 micromol/l** for most people; reduce target to ≤ 300 micromol/l if tophi, chronic gouty arthritis or flares despite SUA < 360 micromol/l.

Check SUA annually once target level achieved.

≤ 360
Or
 ≤ 300

Choosing a ULT and monitoring

If MAJOR CVD, choose allopurinol, e.g. previous MI, stroke, unstable angina.

If NO major CVD, choose either allopurinol OR febuxostat based on comorbidities/preferences (see BNF for doses – NICE does not specify).

Preventing flares: discuss benefits and risks of taking medication to prevent flares.

First line: colchicine.

Second line: NSAID/low-dose oral corticosteroid (off label) +/- PPI.

Switch agent if target SUA not reached or drug is not tolerated.

Monitoring: **monthly SUA** to guide dose increases until target is reached. **Annual monitoring** thereafter.

Refer!

- Diagnostic doubt.
- Drugs contraindicated.
- Drugs not tolerated.
- Drugs ineffective.
- eGFR < 45 .
- Transplant.
- Considering IL-1 inhibitor.



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